

# Millions of charts in. Audit-grade codes out.

Outsourcing buys speed, but your charts leave your organization and the audit risk stays with you for ten years. Keeping it in-house without automation means staying manual — a coder reviews about 40–50 charts a day. **Martlet AI runs inside your own environment, reviews the full population, and verifies every code against the checks CMS applies at audit** — at 99% precision, with no PHI ever leaving your network.

99%

precision on  
automated codes

95%

of cases closed  
automatically

95%

reduction in  
chart review time

100k+

lives running  
on the platform

## What you actually get.

### Runs on your own infrastructure

On-premise, private cloud, or air-gapped. **No chart shipping, no outsourced coders, no PHI leaving your network** — and every decision recorded for later.

### Reviews every chart

The full population runs through the same checks, whatever the volume. **Every chart comes back supported, flagged for deletion, or routed to your team as an exception.**

### Missed codes captured, with proof

Undercoded HCCs come back **with the chart sentence, the encounter and the signature behind them** — so the revenue you capture holds up at audit, years later.

### Unsupported codes flagged before submission

Codes that don't meet the standard are **flagged for removal before they go out**, not discovered after the fact.

# From your data to your submission file.

Charts, claims and prior coding go in. Verified codes and submission-ready files come out. **One system, with no hand-offs to vendors or spreadsheets in between.**

1

## Ingest

Claims, EHR notes, scanned PDFs and bundled files — normalized and de-duplicated.

2

## Prioritize

Charts ranked by expected RAF impact and documentation strength.

3

## Validate

Every code checked against the record, with the evidence linked to its page.

4

## Review

Exceptions routed to your coders, with the evidence already attached.

5

## Submit

Adds and deletes exported in the format your submission pipeline expects.

### 3 Inside validation — verify, then add or delete

Validation doesn't return a pass or a fail. Every code comes back with **its full evidence mapping** — the supporting sentence, the encounter and date of service, the signing provider and their credentials, and the source page. That mapping sorts each code one of two ways.

#### Add — capture revenue

Documented but never coded. Returned **ranked by RAF impact**, with the evidence attached, so a coder confirms rather than goes hunting for the chart.

#### Delete — reduce risk

Problem-list carryovers, unsupported specialties, missing signatures. **Flagged for removal with the reason**, before submission rather than at reconciliation.

On a 12,400-chart run: **11,780 closed automatically** · **620 routed to reviewers** · **deltas exported** — every one with page-level evidence attached.

# What gets checked on every chart.

These are the same checks RADV applies at audit — **run here first, before submission.**

## 01 Document and encounter type

Every encounter inside a bundled file is identified and classified on its own.

## 03 Provider credentials

Signing clinicians are resolved against the current CMS specialty list. Gaps are flagged.

## 05 Labs, imaging and pathology

Diagnostic reports are assessed under the criteria that apply to each type.

## 02 Signature validity

Each record is checked for a valid signature, attributable to the clinician who wrote it.

## 04 Dates of service

A date of service is established per encounter and checked against the period that applies.

## 06 Clinical support

Each diagnosis is checked for support in the encounter itself, not carried forward.

CMS guidance changes. We update the checks to match, so validation follows the rules currently in force.

# What changes for your coding team.

The first pass is done before they open the queue. **Their time goes to judgment calls, not to charts that don't need them.**

## They review exceptions, not everything

95% of cases close before a coder sees them. The rest arrive ranked, with the evidence already linked. **That is what cuts review time by 95%.**

## Coder agreement is tracked

Confirmation and deletion rates are tracked for each coder and reviewer, so **you can see where they disagree** without re-reading closed charts.

## Coders check each other's work

Second-level review and QA sampling run inside the same system. **Every edit is recorded against the coder who made it**, along with the evidence at the time.

## Documentation gaps are grouped by provider

Recurring documentation problems are grouped by clinician and provider group, so you can **address them with the provider** instead of correcting the same issues every cycle.

# Code it right once. The audit is already answered.

Every chart closed here is verified against the same checks CMS uses during a RADV audit. **If your contract is selected, the evidence packets already exist.** [See RADV readiness →](#)

**In production** at health systems, plans and platforms.



## How the engagement works.

### An annual license

You pay a license fee for the year. **Not per chart, and not a percentage of the RAF you capture.**

### Priced to your size

The fee is based on how many contracts and lives you run, and **stays fixed for the year.** Add volume without renegotiating.

### Configured to your workflow

We set the chase list logic, review levels and exports to match how your team works, and **connect to the systems they already use.**

## Bring 500 charts. We'll close them while you watch.

Chase list built, charts verified at 99% precision, exceptions queued, adds and deletes ready to submit — on your charts, inside your environment.

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